

Vextasone

4mg Dexamethasone tablet

Ingredient(s):

Each tablet contains:

Dexamethasone 4mg

Pharmacodynamic:

1. Dexamethasone is a synthetic glucocorticoid and has potent anti-inflammatory properties. It is indicated for the treatment of endocrine disorders, rheumatic disorders, collagen diseases, dermatologic diseases, allergic states and etc.
2. Dexamethasone can decrease or prevent tissue responses to inflammatory processes, thereby reducing development of symptoms of inflammation without affecting the underlying cause. Dexamethasone inhibits accumulation of inflammatory cells, including macrophages and leukocytes, at sites of inflammation. It also inhibits phagocytosis, lysosomal enzyme release, and synthesis and/or release of several chemical mediators of inflammation.

Pharmacokinetics:

Dexamethasone is readily absorbed from the gastrointestinal tract. Its biological half-life in plasma is about 190 minutes. Binding of Dexamethasone to plasma proteins is less than most of the other corticosteroids.

Indication(s):

Dexamethasone is indicated for the treatment of allergic disorders such as bronchial asthma and allergic dermatitis, dermatological diseases such as pemphigus, systemic lupus erythematosus (but not scleroderma), gastrointestinal disorders such as ulcerative colitis, hematologic disorders such as hemolytic anemia, leukemia and thrombocytopenia, neoplastic disorders such as cerebral neoplasm, cerebral edema associated with primary or metastatic brain tumor, eye disorders such as temporal arteritis and rheumatoid disorders.

Dexamethasone is also used to prevent postoperative nausea and vomiting and may be used to manage nausea and vomiting in palliative care.

It is also indicated for secondary adrenal insufficiency due to insufficient corticotrophin secretion. For primary adrenal insufficiency states, such as Addison's disease or after adrenalectomy, hydrocortisone and fludrocortisone in combination are more appropriate. It is also used to diagnose Cushing's syndrome.

Dosage and Administration:

The dose of dexamethasone varies depending on the diseases being treated. A usual dose of 0.5mg to 10mg daily is given for oral administration.

The therapy duration depends on the clinical response of the patient. After a favorable initial response is noted, the dosage should be adjusted to be minimum required to maintain the desired response. It is recommended to withdraw dexamethasone gradually at completion of treatment.

Route of Administration:

For oral administration only.

Contraindication(s):

Dexamethasone is contraindicated in patients with systemic fungal infections and those who have shown hypersensitivity to any component of this product.

Precaution(s) / Warning(s):

1. The adverse effects of Dexamethasone are almost always due to its use in excess of normal physiological requirements. They should be treated symptomatically, where possible the dosage is reduced or the drug slowly withdrawn.
2. Dexamethasone may mask some signs of infections and new infections that may appear during its use. There may be decreased resistance and inability to localise infections when Dexamethasone is used.
3. Dexamethasone should be used cautiously in patients with ocular herpes simplex, psychic derangements and gastric-duodenal ulcer.
4. Unless considered life-saving, systemic administration of corticosteroids is contraindicated in patients with osteoporosis, psychoses, or severe psychoneurosis, and they should be used only with great caution in the presence of congestive heart failure or hypertension, in patients with diabetes mellitus, chronic renal failure, uremia, and in elderly person.
5. Pheochromocytoma crisis, which may be fatal, has been reported after administration of systemic corticosteroids. Corticosteroids should only be administered to patients with suspected or identified pheochromocytoma after an appropriate risk/benefit evaluation.
6. If symptoms persist, please consult a physician.

Interaction with Other Medicaments:

1. Anticoagulants: Anticoagulants response may be reduced by corticosteroids. Thus, it is recommended to closely monitor the coagulation indices.

2. Antidiabetic agents: Dosage adjustment of antidiabetic agents may be required as corticosteroids may increase blood glucose levels.
3. Antifungals: Administration with amphotericin may increase the risk of hypokalaemia.
4. Antacids: Absorption and efficacy of corticosteroids may be decreased with concurrent use of antacids. Dosage adjustment is needed in patients that receive small doses of corticosteroids.
5. Cardiac glycosides: There is an increased risk of arrhythmias due to hypokalaemia in patients receiving cardiac glycosides.
6. Diuretics: Patients may experience excessive potassium loss if potassium-depleting diuretics (such as thiazides and frusemide) or carbonic anhydrase inhibitors (such as acetazolamide) and glucocorticoids are given together.
7. Hepatic microsomal enzyme inducers: Drugs which include hepatic enzyme cytochrome P-450 isozyme 3A4 (e.g., aminoglutethimide, carbamazepine, rifampicin, rifabutin, phenytoin, primidone and phenobarbital) may reduce the therapeutic efficacy of corticosteroids by increasing metabolism rate.
8. Mifepristone: Mifepristone may reduce the effect of corticosteroids for 3-4 days.
9. Non-steroidal anti-inflammatory drugs (NSAIDs): Concomitant use of NSAIDs and corticosteroids may increase the risk of GI ulceration. In patients with hypothrombinaemia, aspirin should be used cautiously in conjunction with corticosteroids. Corticosteroids increase the renal clearance of salicylates and steroid withdrawal may cause salicylate intoxication. Observe patient closely for adverse effects of either medicine.
10. Oestrogens: Effects of corticosteroids may be potentiated by oestrogens. Dosage adjustment of corticosteroid may be needed if oestrogen therapy is commenced or stopped.
11. Somatropin: Corticosteroids may inhibit the growth-promoting effect.
12. Sympathomimetics: If high doses of corticosteroids are administered with high doses of salmeterol, salbutamol, terbutaline or formoterol, risk of hypokalaemia is increased.
13. Vaccines: If possible, routine administration of vaccines should be deferred until corticosteroid therapy is discontinued. The antibody response to vaccines may be diminished in patients receiving corticosteroid therapy.
14. CYP3A inhibitors: Co-treatment with CYP3A inhibitors, including cobicistat-containing products, is expected to increase the risk of systemic side effects. The combination should be avoided unless the benefit outweighs the increased risk of systemic corticosteroid side effects, in which case patients should be monitored for systemic corticosteroid side effects.

Pregnancy and Lactation

The use of Dexamethasone in pregnancy, nursing mothers, or women of childbearing potential requires that the possible benefits of the drug be weighed against the potential hazards, to the mother and embryo or fetus.

Side Effect(s) / Adverse Reaction(s):

1. The prolonged therapy may result in suppression of pituitary-adrenal function and the principal complications resulting from prolonged therapy with Dexamethasone are fluid and electrolyte disturbances; hyperglycemia and glycosuria; increased susceptibility to infections including tuberculosis; peptic ulcers, which may bleed or perforate; osteoporosis; a characteristic myopathy; behavioral disturbances; posterior subcapsular cataracts; arrest of growth; and Cushing's habitus, consisting of "moon face", "buffalo hump", enlargement of supraclavicular fat pads, "central obesity", striae, ecchymoses, acne, and hirsutism.
2. Acute adrenal insufficiency results from too rapid withdrawal of corticosteroids after prolonged therapy. Dexamethasone withdrawal syndrome, consisting of fever, myalgia, arthralgia and malaise.

Symptoms and Treatment for Overdosage, and Antidote(s):

There is no specific antidote for its overdose, therefore, management of the patient consists of symptomatic and supportive therapy.

Shelf-Life: 3 years from the date of manufacture.

Storage Condition(s):

Store at temperature below 30°C. Protect from direct sunlight, heat and moisture.

Product Description & Packing(s):

A white color round tablet, one side impressed with a score and the other side with a mark of "S".



Blister packing of 10's x 100



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